



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متدد

إسم المريض ربيعاً : مريم وهيب سلامه بشاي

العنوان : 49 ش 24 عزبه خليل
المعصره - القاهره

الحى / المدينة : المعصره

المحافظة : القاهره

تاريخ تسجيل الإستماره : 26-09-2026

رقم البطاقة : 27709240105447 رقم التليفون : 01229002029

العمر(تاريخ الميلاد) : 24-09-1977 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحه-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

المقر بما فيه
التوقيع: (-----)

الإحصائي الاجتماعي
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمى وتلطيفى [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبى للمريض وإعادة مناظرة المريض
بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبى المتخصص

Name: **مریم وهیب سلامة بشای**
National ID: **27709240105447**



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :
--:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment

الوقت :

--:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تعريف الشخصية

مريم
وهيب سلامة بشاي
٤٩ ش ٢٤ عزبة خليل
المعصرة - القاهرة

١٩٧٧/٠٦/٢٢ ٢٧٧.٩٢٤.٠١.٥٤٤٧

JK6797196

ما هو

٢٠٢٢/٠٦ ٢٧٧.٩٢٤.٠١.٥٤٤٧

حاصلة على دبلوم صناعي

انثى مسيحية ارملة

البطاقة سارية حتى ٢٠٢٩/٠٦/٢٢

١

تعرفني على كيفية!

الخطوة الأولى (أمام المرأة)

النظري إلى الشديين في المرأة مع وضع اليدين في وضع مستقيم على الجانبين ثم وضع اليدين على الوسط.

- تأكيد بالنظر إلى شكل ولون وحجم الثديين، وأنه هو المعتاد بالنسبة لكي دون تغيير أو دون اختلاف بين الثديين.

- لاحظي وجود أي من العلامات التالية بأي من الثديين:

طفح حدوث تجويف تغير في الشكل أو الحجم بروز الأوردة

حدوث تجوف
تغير في الشكل
أو الحجم
بروز الأوردة

تراجع أو تغيير في
الحداد الحليمية
أو الحليمية
تقرحات بالحداد

تجاعيد بالجلد تحول الجلد لما يشبه
قشور السمك

الخطوة الثانية (أمام المرأة)

- إرفعي يديكي إلى أعلى وتأكدي من عدم وجود أي من التغيرات السابقة

الخطوة الثالثة (أمام المرأة)

إفرازات غير طبيعية أو دم من الحلمات اليمنى أو اليسرى
- أثناء رفع يديكي إلى أعلى، لاحظني خروج أي

افرازات غیر طبیعیة او دم

في حال ملاحظة أي من العلامات السابقة توجهي إلى عيادة صحة المرأة بالوحدة الصحية

15335

www.100millionseha.org

التاريخ

رقم التليفون: ٠١٢٤٦٣٨٧٩
الرقم القومي: ٩٥٠١٠٢٣٤٥٦٧٨٩
الاسم: محمد عبد الله
التاريخ: ٢٠٢٤

O

مستهدفة بالتوعية

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مستهدفة بالتوعية والفحص الإكلينيكي

قياس ضغط	قياس السكر العشوائي	مؤشر كتلة الجسم	الطول	الوزن
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*فكرى نفسك باستمرار بموعد الفحص الذاتي شهرياً

	2019	سبتمبر		2019	أغسطس		2019	يوليو
	2019	ديسمبر		2019	نوفمبر		2019	أكتوبر
	2020	مارس		2020	فبراير		2020	يناير
	2020	يونيو		2020	مايو		2020	أبريل
.	2020	سبتمبر		2020	أغسطس		2020	يوليو
	2020	ديسمبر		2020	نوفمبر		2020	أكتوبر
	2021	مارس		2021	فبراير		2021	يناير
	2021	يونيو		2021	مايو		2021	أبريل

- حافظي على قياس السكر والضغط مرة كل ٦ شهور.

إذا كنتِ، فـ سن الزواج أو الاحاب، استشرى بالتظام طبيب تتظيم الأسرة.

- إطمئني، الكشف المبكر يساعد في السيطرة على الأمراض، وزيادة نسب الشفاء.

مؤيد القادري:

بیتھیب الطیب:

المكان المحولة له للتقييم:

المحبة

Patient ID: 96204	Patient Name English: Mariem Wahib Salamah
Age: ٤٨ سنة, ٨ شهر	2026/06/11 12:25:41 PM :Report Date

Women's Health Outreach Program
Bilateral Full field Digital Mammography

Clinical summary: left breast palpable lump

Technique: Medio-lateral oblique and CC views & High resolution ultrasound

Detailed Findings

Mammographic findings:

- Heterogeneous dense breast parenchyma (ACR-c)
- **Left breast:**
 - **Upper outer, inner & central:** irregular shape, indistinct margin heterogeneous dense lesion measuring 7.5 x 4.8 x 9 cm in CC, AP & TS diameters respectively, reaching the nipple, associated with diffuse skin thickening and trabecular thickening
 - **Lower outer quadrant** ill-defined cutaneous heterogeneous density measuring 4.5 cm in length and 1 cm in thickness
 - The whole area of the pathologic process measuring 9 x 6.7 cm
 - No suspicious microcalcifications noted
 - Multiple level I axillary lymph nodes with globular shape, indistinct margin and diffuse cortical thickness
- **Right breast:**
 - Normal breast parenchyma with no gross evidence of dense spiculated lesion or parenchymal distortion.
 - No suspicious microcalcifications noted
 - Normal skin thickness and contour. Normal nipple / areola complexes

Ultrasound findings:

- Heterogeneous breast parenchyma predominantly glandular
- **Left breast:**
 - Diffuse extensive pathologic process involving the central, upper and inner quadrants with irregular shape, spiculated margin hypoechoic solid lesion measuring 4.3 x 6.4 cm, posterior shadowing with prominent vascularity of low resistance arterial waveform pattern in complementary color Doppler study. The lesion is reaching the skin and nipple with features suggestive of nipple involvement
 - Multiple smaller but similar lesions surrounding the index lesion more evident at the upper inner quadrant
 - The LOQ heterogeneous dense lesion in mammography corresponding to cutaneous ulcerated lesion
 - Moderate diffuse skin thickening up to 7 mm and moderate interstitial edema
 - Regarding the axillary lymph nodes
 - More than 5 level I axillary lymph nodes with globular shape, diffuse cortical thickness and effaced hilum; the largest measuring 2.5 x 2 cm
 - One of the level I axillary nodes showing focal cortical thickness with spiculated margin measuring 1.6 x 1.7 cm and effaced hilum?? features could be suggestive of extracapsular extension
 - More than 5 level II pathologic lymph nodes; the largest measuring 11 x 7 mm and indented hilum with another LN measuring 10 x 8 mm with irregular margin?? features could be suggestive of extra-capsular extension

- More than 5 level III infraclavicular lymph nodes with diffuse cortical thickness and effaced hila

▪ **Right breast:**

- Multiple scattered small cystic lesions with clear content suggestive of fibrocystic changes No gross evidence of suspicious cystic or solid lesions
- No retroareolar dilated ducts
- Normal skin and subcutaneous tissue. Normal nipple areola complex
- Non-specific axillary lymph nodes with preserved oval shape, average cortical thickness and central echogenic hilum

Impression:

- **Left breast (BIRADS 5):** locally advanced diffuse extensive pathologic process with skin and nipple involvement and multi-level pathologic lymph nodes as described (**BIRADS 5**)
- **Right breast (BIRADS 2):** mild fibrocystic changes

Recommendation:

Ultrasound guided true cut biopsy from the left breast lesion and histopathological correlation

Dr. Lamiaa Elsharkawy, MD.

الرقم الوطني:	12133
اسم المريض:	مريم وفيق سلامة
تاريخ الميلاد:	24/09/1977
الرقم الطبي:	null

الرقم الوطني:	12138
اسم المريض:	مريم وافي سلامة
تاريخ الميلاد:	24/09/1977
الرقم الطبي:	null

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اسم المريض:	مريم وفيق سلامة
طريقة العلاج:	ساق
الرقم الوطني:	24/09/1977
اللقب:	ridhi

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الرقم الوطني:	24/09/1977
اللقب:	ridhi

Day Care Nursing Initial Assessment
(Adult and Pediatric)

Patient Name:
MRN:

Abuse & Neglect Screening

Does patient have signs of abuse and neglect?

Yes

No

Pain Assessment

Patient expresses presence of pain: ☐ Yes ☒ No

Pain Location	Character (Use codes below)	Radiation	Pain intensity (use scales below)	Frequency
None	2	None	3	unlabeled

Character Codes = Document Term Best Describing Pain:

- Sharp
- Dull
- Stabbing
- Sore
- Crushing
- Deep
- Colic
- Aching
- Throbbing
- Tight
- Shooting
- Pressing
- Squeezing
- Pulling

Numeric scale (adults):

0	1	2	3	4	5	6	7	8	9	10
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Wong Baker Faces (pediatric):



Fall Assessment

Adults: Use Morse Fall Risk Scoring, Pediatric: use Humpty Dumpty Scoring

Oncology Assessment

Cycle no.: 4 Cycle day: 2

Arm precautions: ☒ None ☐ Right arm ☐ Left arm

Apply arm precaution bracelet if present

Central Venous Access Assessment (if applicable)

Central Venous Access Device (CVAD) details

CVAD Type: ☒ Intravenous canula, size: 1.2 ☐ Proximal ☐ Distal ☐ Medial ☐ PICC ☐ Port ☐ Hickman

Considerations

Device intact? ☒ Yes ☐ No

Skin intact? ☒ Yes ☐ No

Signs of localized site infection? ☒ Yes ☐ No

Dressing changed? ☒ Yes ☐ No

*Label dressing with date and time and nurse initials

Extravasation Risk Factors:

Patient related factors:

- ☐ Small/ fragile veins
- ☐ Bleeding abnormalities
- ☐ Obesity
- ☐ Sclerosed veins from previous chemotherapy sessions

Consider central line according to the above risk factors

Cannulation & Infusion procedure related:

- ☐ Multiple attempts at cannulation
- ☐ Prolonged infusion

Monitoring During Infusion

Time (every 30 mins)	Check cannulation site	Time (every 30 mins)	Check cannulation site
5:30 PM	Good	6 PM	Good
6:30 PM	Good	7 PM	Good

Day Care Nursing Initial Assessment (Adult and Pediatric)

Patient Name:
MRN:

Action	Done	Details
Remove line		
Mark the extravasation site with a marker suitable for use on skin		
Describe appearance of PIVC or Huber needle if removed		
Drug/ ointment application as per doctor order		Details of drug/topical treatment administered:
		Time of administration:
Measure the extravasation site		Diameter (cm): _____ Length(cm): _____ Width(cm): _____
Inform physician to consider medical imaging		
Administered analgesia as per doctor order		
Education provided to patient		Required teaching points: ☐ Sign/symptoms of necrosis ☐ Sign/symptoms of sensory loss ☐ Monitoring temperature ☐ Topical care ☐ Protection from sunlight ☐ Other: _____
Teaching aids used: ☐ Written ☐ Verbal ☐ Demonstration		
Complete an incident report		
Vital Signs After Infusion		
Temp.	Pulse	BP.
37	80	110/70
		Respiratory Rate
		19
Discharge & Patient Education		
Considerations	Yes	No
Frequency given for CVAD dressing change?		Frequency: Start Date: _____ Location: ☐ GRI-E ☐ Other Hospital ☐ Home/Self care
Has intravenous cannula/Huber needle been flushed & removed?		
Did the patient receive oral medications upon discharge?		
1- Patient has been educated about oral medications treatment plan?		
2- Is the patient able to read the drug label and provided information?		If no, inform treating physician

جمهورية مصر العربية
بطاقة تعريف الشخصية



مريم
وهيب سلامة بشاي
٤٩ ش ٢٤ عزبة خليل
المعصرة - القاهرة



٢٧٧.٩٢٤.١.٥٤٤٧

JK6797196



Patient's Name:

Marjem wahren salama

Date:

2026-08-20

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

Pelvi-abdominal ultrasonographic examination, revealed:

- The liver is not enlarged measuring about 15 cm with homogeneous echo-pattern. No definite focal lesions or IHRD by US criteria.
- Patent portal vein with average diameter showing normal flow direction.
- Gall bladder is averagely dilated with normal wall thickness. No stones or mud seen. Non-dilated CBD.
- Spleen is not enlarged measuring about 10 cm with homogenous echo pattern and no definite focal lesions.
- The midline structures and visualized portions of the pancreatic head are free.
- Colonic gaseous distension.
- Both kidneys are average in size, shape, echo-pattern and differentiation. No backpressure changes.
- No ascites at time of examination.
- Average filling of the urinary bladder with no stones or masses.
- Normal size of the uterus with no definitive focal lesion.
- No gross adnexal lesion.
- No pelvic fluid collection.

Conclusion:

- Colonic gaseous distension.
- Otherwise, unremarkable study.

Best regards;

Dr. Mohammad Islam



MINISTRY

قسم الباثولوجي

الهيئة العامة للمستشفيات والمعاهد التعليمية
المعهد القومي للكبد والجهاز الهضمي
والأمراض المعدية

PATHOLOGY DEPARTMENT HISTOPATHOLOGICAL REPORT

Patient name: مريم و طيب سلامة	Age: 49 years	Sex: female
Request type: ميلد	Sub request type:	
Hospital no:	Pathology no.: 499B-2026	
Department: صحة مرآة	Date of result: 23/6/2026	
Referred by: د/مروى حسن	Date of biopsy: 16/6/2026	
Nature of specimen: tru-cut biopsy	I.D. Number: 27709240105447	
Clinical diagnosis: Left breast mass		
<u>Gross pathology :</u>		
multiple gray white tissue cores measured 1cm, totally submitted		
<u>Immunohistochemical Technique:</u> Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against:		
ER- PR- Her2- Ki67- Ecadherin		
<u>Microscopic picture:</u> Section examined revealed breast core tissue showed clusters, nest, cribriform pattern of anaplastic ductal cells with marked anaplasia and poor tendency to form tubules dispersed among desmoplastic stroma with mild lymphocytic response. There are positive lymphovascular tumor emboli but no evident perineural infiltration. No ductal carcinoma insitu is seen.		
<u>Immunohistochemical Result:</u>		
E cadherin : Diffuse positive		
ER: Strong nuclear staining in 90% of tumor cells (Allred Score 8/8).		
PR: Strong nuclear staining of 80% of tumor cells (Allred Score 8/8).		
Her2: Moderate complete membranous staining of >10% tumor cells (score 2+) <u>Equivocal for SISH</u>		
Ki67: Positive nuclear staining of 70% of tumor cells.		
<u>Diagnosis:</u> Right breast mass: Tru Cut Biopsy:		
Invasive Duct Carcinoma (NST), Grade III		

Signature:

Dr.Mona Mohamed.M.D

M

Pathology Lab. Manager

BASMA REFAAT M.D

Patient Name	مریم و طیب سلامه	Case No.	344578
Sex/Age	Female 48 Y	Patient No.	25264
Registered Date	28/7/2026 10:52:04 AM	Referred By Dr.	Out Patient
Validation Date	16/8/2026 12:08:21 PM	Branch	Gustave Roussy
Reporting Date	20/8/2026 09:11:55 AM	Pathology No.	SR-857/26

Pathology Report

Clinical Data: Left breast mass was diagnosed elsewhere as invasive duct carcinoma, Grade 3.

Nature of Specimen:

- Only one block labelled by (499B-26) was referred for immunophenotyping (SISH). (Tru-cut biopsy).

Technique:

- Sections of the unstained slides, processed in Benchmark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against: SISH.

Results:

* IPT (All controls showed appropriate reactivity):

- SISH: Not amplified.

(Average Her2 copy No.: (3.75), Her2/CEP17 ratio: (1.9)).

Diagnosis:

Left breast mass, referred block, Tru-cut biopsy:

IPT:

- SISH: Not amplified, Group V.

(Average Her2 copy No.: (3.75), Her2/CEP17 ratio: (1.9)).

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.



غرفة رقم (44) - مخبر النور - مبنى القبة - مخبر القبة - المخبر 4 - مصر
 Room No. (44) - First Floor-Building No. 9/87- Nile Corniche - Mitter El Qadema - Cairo, Egypt

- More than 5 level III infraclavicular lymph nodes with diffuse cortical thickness and effaced hila

▪ Right breast:

- Multiple scattered small cystic lesions with clear content suggestive of fibrocystic changes No gross evidence of suspicious cystic or solid lesions
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- Normal skin and subcutaneous tissue. Normal nipple areola complex
- Non-specific axillary lymph nodes with preserved oval shape, average cortical thickness and central echogenic hilum

Impression:

- Left breast (BIRADS 5): locally advanced diffuse extensive pathologic process with skin and nipple involvement and multi-level pathologic lymph nodes as described (BIRADS 5)

- Right breast (BIRADS 2): mild fibrocystic changes

Recommendation:

Ultrasound guided true cut biopsy from the left breast lesion and histopathological correlation

Dr. Lamiaa Elsharkawy, MD.

Patient ID: 96204	Patient Name English: Mariem Wahib Salamah
مریم أ. سالمه Age:	2026/06/11 12:25:41 PM : Report Date

Women's Health Outreach Program Bilateral Full field Digital Mammography

Clinical summary: left breast palpable lump

Technique: Medio-lateral oblique and CC views & High resolution ultrasound

Detailed Findings

Mammographic findings:

- Heterogeneous dense breast parenchyma (ACR-c)
- Left breast:
 - Upper outer, inner & central: irregular shape, indistinct margin heterogeneous; dense lesion measuring 7.5 x 4.8 x 9 cm in CC, AP & TS diameters respectively, reaching the nipple, associated with diffuse skin thickening and trabecular thickening
 - Lower outer quadrant ill-defined cutaneous heterogeneous density measuring 4.5 cm in length and 1 cm in thickness
 - The whole area of the pathologic process measuring 9 x 6.7 cm
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 - Multiple level I axillary lymph nodes with globular shape, indistinct margin and diffuse cortical thickness
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 - Normal breast parenchyma with no gross evidence of dense spiculated lesion or parenchymal distortion.
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Ultrasound findings:

- Heterogeneous breast parenchyma, predominantly glandular
- Left breast:
 - Diffuse extensive pathologic process involving the central, upper and inner quadrants with irregular shape, spiculated margin hypoechoic solid lesion measuring 4.3 x 6.4 cm, posterior shadowing with prominent vascularity of low resistance arterial waveform pattern in complementary color Doppler study. The lesion is reaching the skin and nipple with features suggestive of nipple involvement
 - Multiple smaller but similar lesions surrounding the index lesion more evident at the upper inner quadrant
 - The LOQ heterogeneous dense lesion in mammography corresponding to cutaneous ulcerated lesion
 - Moderate diffuse skin thickening up to 7 mm and moderate interstitial edema
 - Regarding the axillary lymph nodes
 - More than 5 level I axillary lymph nodes with globular shape, diffuse cortical thickness and effaced hilum; the largest measuring 2.5 x 2 cm
 - One of the level I axillary nodes showing focal cortical thickness with spiculated margin measuring 1.6 x 1.7 cm and effaced hilum?? features could be suggestive of extracapsular extension
 - More than 5 level II pathologic lymph nodes; the largest measuring 11 x 7 mm and indented hilum with another LN measuring 10 x 8 mm with irregular margin?? features could be suggestive of extra-capsular extension

Patient's Name: Maryam Waheb Salama Bishay
Date: 2026-07-19
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST CT EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS

REVEALED:

I- CHEST:

- Known case of right breast cancer with skin thickening & edema beneath it a left breast mass lesion (6.8x3.5cm) encroaching upon the chest wall pectoral muscle with another lateral breast mass measuring (28x21cm) with left axillary enlarged rounded lymphnodes with the largest measuring (1.8x1.3cm)
- Left Sided posterior and basal pleural thickening with mild pleural effusion
- Clear right lung field with no focal lesions or nodules & No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pericardial collection.
- The cardiac size and great vessels appears normal.

II- ABDOMEN AND PELVIS:

- Average Sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal size, average size, and showing a smooth, regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, aorta, and IVC.
- The urinary bladder shows no evidence of gross masses, diverticulac, or stones.
- Noted left ovarian cyst measuring (4.0x3.0cm) with multiple thin internal septations for Ultrasound assessment yet the rest pelvic organs appear Normal with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Noted Diffuse decreased bony density with diffuse bony sclerosis

Opinion:

- Known left breast cancer with left Axillary Lymphadenopathy.
- Left Sided posterior & basal pleural thickening & mild pleural effusion.
- Noted Diffuse decreased bony density with diffuse bony sclerosis
- Noted left ovarian cyst measuring (picture of Hemorrhagic cyst) for U/S assessment.

Best regards,

Dr. Ayman Shawky, FEBR

Dr Ayman Shawky

Dr Ayman Shawky



SMART SCAN
سمارت سكان للشمعة

Name: مريم وهيب سلامة Age: 47 Years ID : 135635
Referred By Prof Dr: بيتر سمير حكيم
جهة التحويل:

branch: Imbaba 09/06/2026 18:36:32

MRI EXAMINATION OF THE LUMBOSACRAL SPINE **REVEALED:**

Technique:

- Multiple pulse sequences taken in multiple planes. No contrast was given.

Findings:

- Reduced lumbar lordosis.
- Diffuse heterogeneous marrow signal with scattered illdefined lesions of low T1 & high T2 signal .
- Decreased height of L5 associated with bowing of its posterior border seen indenting the theca
- Normal diameters of the bony spinal canal.
- No significant disc lesions
- Normal MRI appearance of the conus medullaris and cauda equine nerve roots. No intra spinal lesions seen.
- No abnormality detected in the para-spinal soft tissues.

Opinion:

- Diffuse heterogenous marrow signal with scattered illdefined lesions most likely metastatic for clinical correlation.
- L5 vertebral body collapse.

Dr:Hamed Jamal,MD .Radiology Consultant



SMART SCAN
سمارت سكان للاشعة

Name: مريم و هيب سلامة Age: 47 Years ID : 135635
Referred By Prof Dr: بيتر سمير حكيم
جهة التحويل:

branch: Imbaba 09/06/2026 18:37:09

Plain Radiographic Examination of Pelvis Revealed:

- patchy areas of bone mottling involving the pelvic bones & both proximal femurs for clinical correlation & further assessment.
- Intact both sacroiliac joints showing smooth regular cortical outline of the articular surfaces and preserved joint spaces.
- Intact both hip joints showing smooth regular cortical outline of the articular surfaces and preserved joint spaces.

Dr: Eman Mohamed Abdel Moneim, MSC. Radiology Consultant



مبادرة رئيس الجمهورية
لدم صحة المرأة المصرية



08/30/2026

MDT FORM

Name:	مريم ربيع مطلق شاتي	MRN:	12133
Age:	49 Y	ID:	27709240105447
Menopausal Sta		Phone:*	01275334475
NRS SCORE:		Marital Status:	
BMI:		Family History:	+VE (AUNTE)
Gender		Reason Of MDT:	NEW CASE
CO Morbidity:	FREE	ECOG Performance Status	00 10 20 30 4
Surgical History:*	3 CS		

Clinical Data:

BX: IDC G 3

ER: 8/8 PR: 8/8 HER2: -VE KI67: 70%

History:

CLINICAL EXAMINATION
LT BREAST PUNGATING ULCERATING FIXED MASS
T4D N3 M1

Sonomammography:

ACR C
LOCALLY ADVANCED DIFFUSE EXTENSIVE PATHOLOGIC PROCESS WITH
SKIN AND NIPPLE INVOLVMENT WITH MULTIPLE LVEL PATHOLOGICAL LN
Biomarker/Molecular

Met Workup:

**CT CAP
LT SIDED POSTERIOR BASAL PLEURAL THICKENING AND MILD PLEURAL
EFFUSION
DIFFUSE DECREASED BONY DENSITY WITH DIFFUSE BONY SCLEROSIS

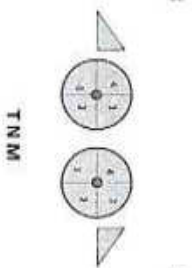
**MRI LUMBOSACRAL
SCATTERED ILL DEFINED LESIONS LIKELY METASTATIC
L5 VERTEBRAL BODY COLLAPSE

Recommendations:

FOR BONE SCAN
NACT

MDT Outcome & Plan

taxol carboplatin weekly then maintenance hormonal treatment



Right Side Left Side
Q1 Q2 Q3 Q4 Q1 Q2 Q3 Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. ربيع مطلق شاتي	د. محمد خير لافي		

(Handwritten signature)